

Leukemia Clinic - Progress Note

Pt: Rodriguez, Nancy

MRN: AML020

DOB: 27-Nov-1962

DOS: 28-Mar-2024

Dx: AML, intermediate-risk (ELN 2022), WT for NPM1/FLT3/IDH on 18-Nov-2022.

Rx: Venetoclax (VEN) + Azacitidine (AZA) since 25-Nov-2022

S: Pt reports good energy, no new c/o. Denies f/c/ns, bleeding, bruising. Tolerating C17 VEN/AZA well. Adherence to VEN 400mg D1-14 sched confirmed. Notes mild constipation, managed w/ Senna. No signs of TLS.

O:

Vitals: T 36.9, BP 128/76, HR 68, Wt 71 kg.

Exam: NAD. No LAD, HSM. Skin clr.

Labs (28-Mar-24):

- WBC 3.1 (prv 2.9)
- ANC 1.5 (prv 1.3)
- Hgb 11.8 (prv 11.5)
- Plt 141 (prv 135)
- Uric Acid 4.8, Cr 0.8, LFTs WNL.

A/P:

61 y/o F w/ intermediate-risk AML on long-term VEN/AZA therapy, now 16 mos post-dx.

- **Disease Status:** Remains in CRi (complete remission with incomplete count recovery), sustained. BMBx 4 mos ago was MRD-neg by flow. Counts remain stable in a low-normal range, which is expected w/ this regimen. No evidence of progression.
- **Therapy Plan:** Continue current regimen. She is due for AZA today (C17D5).
 - Dispense script for VEN 400mg, D1-14 for Cycle 18.
 - No dose mods needed for myelosuppression at this time; counts are stable.
- **Prophylaxis:**
 - Continue Valacyclovir 500mg daily.
 - Continue Posaconazole 300mg DR tab daily. Check level next visit.
- **Comorbidities:**
 - **CAD:** Stable. Continue ASA 81mg, Metoprolol, Rosuvastatin.
- **FU:** RTC in 1 wk for C17D12 CBC check. Then again in 3 wks for C18 initiation.

B. Carter, MD / J. Mills, NP